

Public Health Emergency Operations Center "COUSP DRC"

MVE-17 Incident Management System

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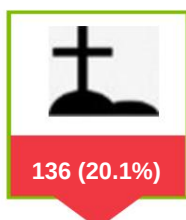
Situation Report of the 17th Ebola Virus Disease Outbreak / DRC

SitRep N°27/MVB_10/06/2026

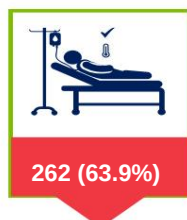
Affected provinces (3)	ITURI, NORTH KIVU & SOUTH KIVU
Affected Health Zones (29)	• Ituri (19/36): Aru, Aungba, Bambu, Bunia, Damas, Gety, Kilo, Komanda, Lita, Logo, Mambasa, Mangala, Mongbwalu, Nizi, Nyankunde, Rimba, Rwampara, Tchomia, Kambala. • North Kivu (9/34): Beni, Butembo, Goma, Kalunguta, Katwa, Kyondo, Oicha, Masereka and Vuhovi. • South Kivu (1/34): Miti-Murhesa
Reporting date	June 10, 2026
Publication date	June 11, 2026



Total confirmed cases
for the 3 provinces



Cumulative deaths
among confirmed cases
and case fatality rate



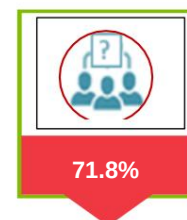
Patients in
isolation -
hospitalization



Cumulative number
of recoveries



Suspected cases of
the day



Contact tracing rates
for the 3 provinces

*Data is being harmonized to include confirmed cases still in the community as well as cases of tax evasion in the overall count

Suspicious deaths occurring in the community are subject to investigation.
further investigation with a view to possible reclassification as probable cases.

1. HIGHLIGHTS

- As of June 10, 2026, **41 new confirmed cases, including 9 deaths**, were reported in the health zones of Bunia (12), Mongbwalu (5), Rwampara (4), Bambu (2), Nizi (2), Lita (1), Nyakunde (1) Mangala (1), Kambala (1) in Ituri and Beni (4), Katwa (4), Butembo (1), Kyondo (1), Masereka (1), Vuhovi (1) in North Kivu.
- **Three new health zones affected**, including 2 (**Masereka and Vuhovi**) in North Kivu and 1 (**Kambala**) in Ituri
- **Two (2) confirmed** Ebola virus disease patients in the Bunia health zone (Ituri province) have been declared cured.

2. EPIDEMIOLOGICAL AND OPERATIONAL CONTEXT

Ituri province, located in the northeast of the Democratic Republic of Congo, shares a long and porous border with Uganda and South Sudan. For over two decades, it has faced a chronic humanitarian crisis linked to armed conflict and recurring population displacements. Its population is estimated at over 8 million, including more than one million internally displaced persons. The Mongbwalu health zone, considered the starting point of the epidemic, is located in the Djugu territory, 70 km from Bunia. This area is characterized by the presence of armed groups, frequent population movements towards Uganda, and numerous artisanal mining sites attracting migrant workers. In September 2018, Ituri province had already been affected by the 10th Ebola virus disease (EVD) outbreak, which was then raging in North Kivu. The current epidemic, caused by the Bundibugyo ebolavirus, was officially declared on May 15, 2026, by the Minister of Public Health. The three affected provinces have a population

total estimated at nearly 15 million inhabitants, with massive internal displacements and cross-border flows to neighboring countries.

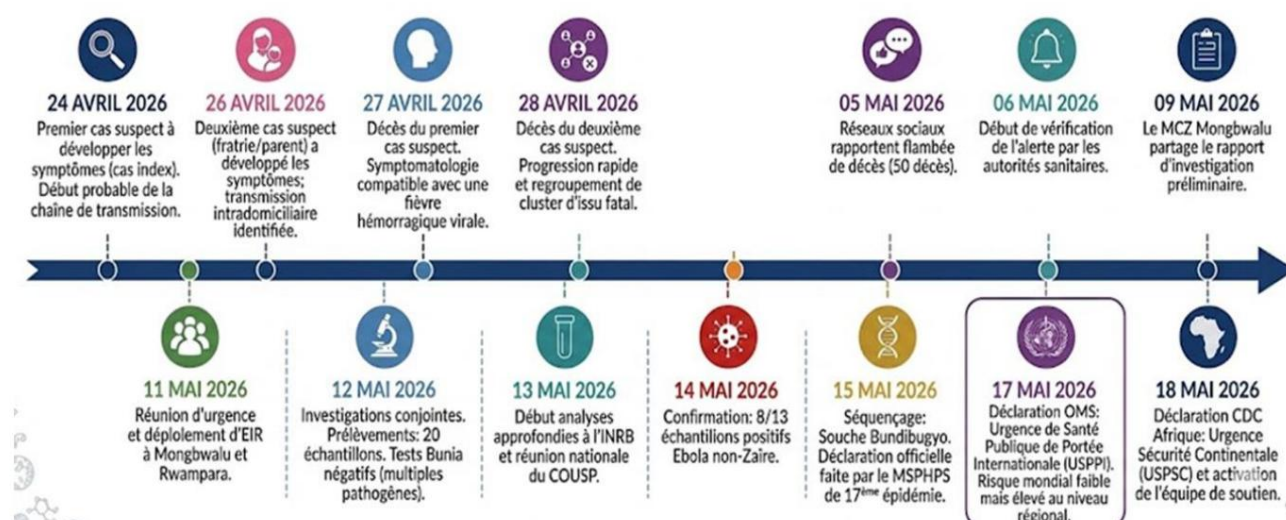


Figure 1. Chronology of events of the Ebola-Bundibugyo virus disease epidemic.

3. DETAILED EPIDEMIOLOGICAL ANALYSIS

3.1 Spatial distribution and active hotspots as of June 10, 2026

Province	Confirmed cases	Deaths (confirmed)	Lethality	Affected health zones	New confirmed cases (24h)
Ituri	629	109	17.3%	19 out of 36 (52.8%)	29
North Kivu	44	26	59.1%	9 out of 34 (26.5%)	12
South Kivu	3	1	33.3%	1 out of 34 (2.9%)	0
Total	676	136	20.1%	29 out of 104 (27.9%)	41

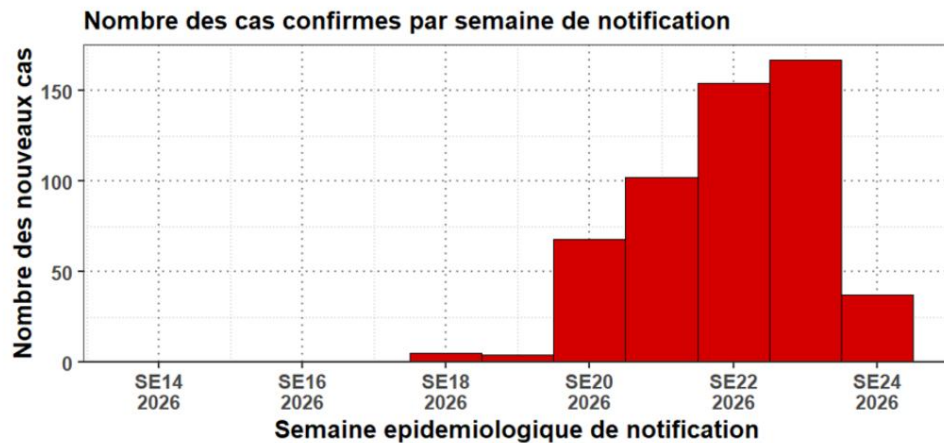
Three new health zones have been affected: Masereka and Vuhovi (North Kivu) and Kambala (Ituri). Overall, the number of affected health zones has increased from 26 to 29 out of 104 (section 3.1). As of June 10, 2026, Ituri province accounted for 93.0% of the confirmed cases reported in the three affected provinces. North Kivu province remains the most affected in terms of case fatality rate (59.1%), while Ituri province maintains a low case fatality rate (17.3%). South Kivu province recorded its last confirmed case on May 26, 2026.

3.2 Distribution by health zone (Cumulative as of 10/06/2026)

- **Ituri Province (629 cases):** Bunia (185), Rwampara (137), Mongbwalu (132), Nyankunde (33), Rimba (3), Mambasa (2), Bambu (7), Aru (3), Damas (3), Kilo (4), Nizi (7), Mangala (2), Aungba (2), Gety (1), Komanda (3), Lita (6), Logo (2), Tchomia (2) and Kambala (1), (Ninety-four (94) additional cases are being ventilated).
- **North Kivu Province (44 cases):** Katwa (18), Beni (9), Butembo (9), Oicha (2), Kalunguta (1), Kyondo (2), Goma (1), Masereka (1) and Vuhovi (1).

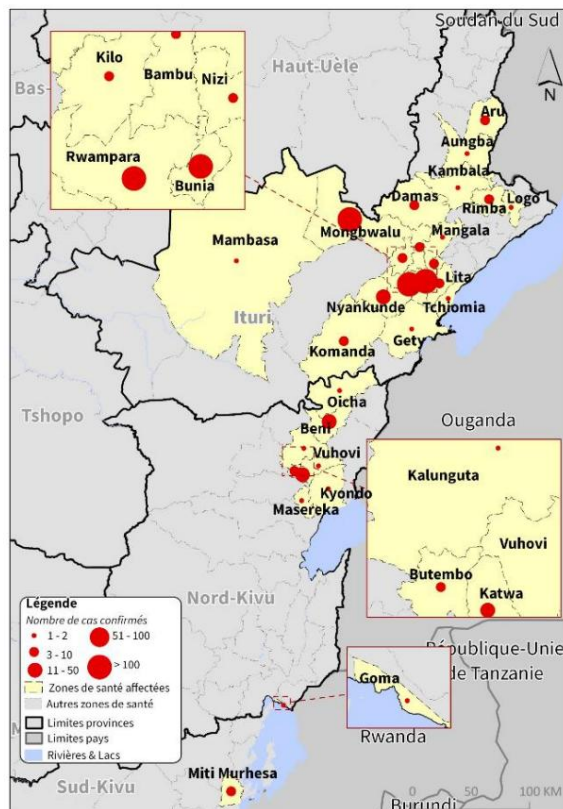
- South Kivu Province (3 cases): Miti-Murhesa (3).

3.3 Time analysis: The number of confirmed cases is increasing week by week, indicating continued transmission of the disease within the community. A rapid geographical expansion of the epidemic is a real possibility if public health measures are not implemented quickly.



Source : DHIS2 Tracker - Riposte MVE, juin 2026. Données incluses dans l'analyse : n cas confirmés = 537.

3.4 Epidemic mapping



Figures 2 & 3. Spatial distribution of confirmed Ebola cases by affected health zone and in the three affected provinces as of June 10, 2026

Table 1. Distribution of confirmed cases and deaths of Bundibugyo Ebola virus disease by health zone and province, Democratic Republic of Congo, as of June 10, 2026.

Province / Health Zone	Cumulative confirmed cases (n)	Cumulative confirmed deaths (n)	Case fatality rate (%)
ITURI			
Bunia		17	9.2%
Rwampara	185,137	25	18.2%
Mongbwalu	132	49	37.1%
Nyankunde	33		3.0%
Bamboo	7	1	28.6%
Aru	3		33.3%
Kilo	4		25.0%
Nizi	7	2	0.0%
Mangala	2	1	0.0%
Damascus	3	1	0.0%
Aungba	2		50.0%
Gety	1	0	0.0%
Komanda	3	0 0 1 0 0	0.0%
Lita	6	0	0.0%
Logo	2	0	0.0%
Mambasa	2		50.0%
Rimba	3	1	0.0%
Tchomia	2	0	0.0%
Kambala			100.0%
Other ZS (undisclosed data)	1 94	0 1 10	10.6%
Ituri subtotal	629	109	17.3%
NORTH KIVU			
Katwa	18	10	55.6%
Blessed	9	7	77.8%
Butembo	9	5	55.6%
Oicha	2	2	100.0%
Kalunguta			100.0%
Kyondo	1	1	0.0%
Goma		0	0.0%
Masereka		0	0.0%
Vuhovi			100.0%
North Kivu subtotal	2 1 1 1 44	0 1 26	59.1%
SOUTH KIVU			
Miti-Murhesa	3	1	33.3%
South Kivu subtotal	3	1	33.3%
TOTAL	676	136	20.1%

4. RESPONSE ACTIONS BY PILLAR

4.1 Coordination

National / Ituri:

- Organization of ceremonies for **two (2)** patients cured at the CTE Elikya (ZS Bunia).
- Deployments of national experts to the North Kivu DPS in Beni-Butembo and to the Ituri DPS towards the Mongbwalu ZS and surrounding areas.

4.2 Epidemiological surveillance

Table 2. Management of epidemiological alerts (24h) as of June 10, 2026

Indicator		Ituri	North Kivu	South Kivu	Total
Alerts raised		143	107	7	257
Alerts investigated		116	107	7	230
Investigation rate		81.1%	100.0%	100.0%	89.5%
Alerts confirmed	Alive	60	3	6	69
	Deceased	31	19	0	50
Total suspected cases for today		91	22	6	119

As of June 10, 2026, 257 **alerts** had been reported across the three affected provinces, **230 (89.5%)**

These alerts were investigated. Thus, **119 suspected cases (31 deaths) were recorded** in the 3 affected provinces.

Table 3. Contact tracing of confirmed cases as of June 10, 2026

Province	Contacts under follow-up	Contacts viewed	Follow-up rate (%)
Ituri	(n) 4,703	(n) 3,357	71.4%
North Kivu	841	597	71.0%
South Kivu	224	187	83.5%
Total	5,768	4,141	71.8%

- Continued validation activities for identified probable cases.
- Continued capacity building for health center teams and RECO of the health areas visited on the Ebola surveillance and community-based surveillance.
- 260 new contacts were listed, including 151 in Bunia, 41 in Mongbwalu and 67 in the Lita health zone.

Update on checkpoint and point of entry (PoC/PoE) activities

Table 4. Monitoring of indicators at Points of Entry and Points of Control (PoE/PoC) as of June 10, 2026

Indicators	Results
Proportion of activated PoCs	48.6% (17/35)
Number of past travelers	31,413

% of travelers screened % of	98.2%
travelers who washed their hands	98.3%
% of travelers made aware	97.1%
Number of alerts notified	3
Number of validated alerts	3
Percentage of alerts investigated or bodies recovered within 24 hours;	0%
percentage of results available within 24 hours	0%
Number of alerts transferred to the CT/CTE or for EDS	0

4.3 Laboratory

- **Ituri:** 109 new samples collected and analyzed (positivity 26.6%; n=29)

4.4 Infection Prevention and Control (IPC)

- **Ituri:** Day 3 of IPC training with WHO support for 50 healthcare providers (ITs and focal points from 24 health centers in the Bunia health zone). IPC and EDS training began with support from partner FHI360 for the IPC pillar (Nyakunde health zone). A briefing was held for 17 journalists from the Ituri press on preventive measures during this Ebola outbreak. In the Bunia health zone, decontamination of two households of confirmed cases (Q/RVA2/Bankoko health center) and (Ngezi health center), the Anuarite medical center, the Mudzi Maria health center, the Elikya hospital, and finally, a briefing for 8 people from a household on handwashing. A briefing was also held for 4 community health workers on standard precautions/ICP for breaking transmission chains in the community.
- **North Kivu:** 749 people reached in the Vutsundo and Makasi health districts; 2,817 people sensitized in the Kivika, Makangala, Masuli and Vungi health districts; 25,411 people sensitized and 12,894 households visited in the Goma, Karisimbi and Nyiragongo health zones; 17,145 people reached during mass awareness campaigns and 278 during educational talks in markets, parking lots and checkpoints; 3 educational talks organized in the Kyaghala health district (67 people reached); 45 religious leaders briefed; 80 community leaders trained; 19 members of the Village Savings and Loan Association (VSLA) sensitized in the Kyaghala health district; Exchange session with the priest of Vusenzeru for contact follow-up; 7 community alerts reported by the Community Health Workers (CHWs); 14 additional alerts reported with the involvement of 130 community actors; 9 rumors collected, 6 of which were clarified; 8 local radio stations engaged with the production and broadcast of 3 radio formats; 6 banners received and started to be displayed in the hotspots of Katwa; Strategic orientation meeting with the PNCPS North Kivu coordinator for Katwa and Butembo.
- **South Kivu:** Decontamination of the Muhumba Military Hospital and Cantonment Site Nyakadaka; scorecard evaluation of 2 ESS (Chahoboka: 17.7% and Nzinzi Health Post: 11.3%) with development of a recovery plan; briefing of 40 service providers from the Military Hospital of Muhumba; briefing of 6 hygiene officers from the Kahungu Health Center on the preparation of the 0.05% and 0.5% chlorine solutions for decontamination, and the donning and doffing of PPE; briefing of 6 hygiene officers and 2 supervisors from the Katana Health Zone on decontamination using the 3-bucket technique, body securing, and , EDS; Assessment of 5 hotspots and installation of lava devices main in the Mitimurhesa health zone and Katana health zone.

4.5 Holistic care

- Two (2) confirmed Ebola virus disease patients in the Bunia health zone, at the Elykia Ebola Treatment Center (Ituri province) have been declared cured.
- Death of a child under 5 years old confirmed at the Rwampara Ebola Treatment Center
- Death of the 10-month-old girl from the Saint-Nicolas orphanage who tested positive for CTE CME

Table 5. Patient movement within healthcare facilities (CTE/CT/CI) as of June 10, 2026

Indicator	Ituri	North Kivu	South Kivu	Together
Patients in bed (Day -1)	239	17	6	262
Total admissions (24h)	26	0	0	26
Exits (24h)				
Deceased (Suspects/Confessions)	10	0	0	10
No cases / Cured	13	0	0	13
Escapees (Suspects/Confessions)		0	0	3
Transferred to the HGR	30			
Total outflows	26	0	0	26
Patients in isolation (End J)	239	17	6	262
including confirmed (NC+AC)	126	5	1	132
including suspected	113	12		130
Overall occupancy rate	77.9% (239/307)	23.9% (17/71)	5 18.8% (6/32)	63.9% (262/410)

Mental health activities and psychosocial support

Table 6. Key indicators of mental health and psychosocial support activities as of June 10, 2026

KEY INDICATORS	ITURI	NORTH KIVU	SOUTH KIVU
New confirmed cases admitted to the CTE who benefited from SMSPS interventions	1 (4.2%)	0	0
Confirmed cases currently being monitored that have benefited from SMSPS interventions	53 (52.0%)	4 (100.0%)	1 (100.0%)
New suspected cases that benefited from SMSPS interventions	18 (72.0%)	6 (60.0%)	2 (100.0%)
Suspected cases under follow-up that have benefited from SMSPS interventions	7 (8.0%)	22 (78.6%)	7 (100.0%)
Laboratory results have been announced	10	0	5
Including positive ones	2 (20.0%)	1	0
Number of children separated in daycare who benefited from SMSPS interventions		0	
Number of children separated in the community who benefited from SMSPS interventions	0	7	
Number of accompanying persons at the CTE who benefited from SMSPS interventions	30	10	5
Number of PPLs who benefited from SMSPS interventions	26	14	6

Number of household and community members who benefited from SMSPS interventions	32	5	7
Number of EDS supported	1	4	0

4.6 Continuity of services.

- **Evaluation of health centers in the health zones of Kilo, Bambu and Nizi**
- **Development of a recovery plan for HGR MONGBWALU.**

4.7 CREC

North Kivu: 749 people reached, including 276 men and 473 women, through awareness campaigns on preventive measures against Ebola Virus Disease (EVD) during home visits in AS VUTSUNDO and MAKASI

- 7 community alerts reported by community relays.
- Meeting with the parish priest of Vusenzero to facilitate contact tracing and listing of other contacts within the family of the deceased positive case in AS Vusenzero
- 25,411 people sensitized and 12,894 households visited through door-to-door activities conducted by RECOs and other community actors in the Health Zones of Goma, Karisimbi and Nyiragongo; 18 local radio stations engaged with 3 radio formats produced and broadcast
- 3 educational talks with 67 people reached, including 21 men and 46 women, on preventive measures against MVE in AS KYAGHALA

South Kivu: Briefing and advocacy with 102 teachers from the Kabare II sub-division in the Miti-murhesa health zone on Ebola virus disease and their role in the response against Ebola virus disease; 5,681 people, including 3,215 women, sensitized during home visits in the Kahungu, Chaoboka and Lwiro health areas in 2,126 households visited; 5 broadcasts produced on community radio stations in Katana, Kalehe, Voldi, Enyanya and Maendeleo on prevention measures and signs of Ebola virus disease.

Ituri: A motorized caravan traveled to Bunia, the capital of Ituri province, to raise awareness among the population about Ebola virus disease (EVD) prevention through various practices (handwashing, adherence to temperature checks, blood and casualty incidents, contact tracing, etc.). This motorized caravan covered the main roads of Bunia and part of the Rwampara health zone. Support: Congolese Government (Ministry via INSP/COUSP)

- Guided tour of the Ebola Treatment Center (ETC) in the Rwampara health zone for 30 journalists to prepare broadcasts supporting the Ebola response. Observation: Treatment conditions are improving, according to testimonies from community health workers who visited the ETC. Support: FHI 360

The Ebola Response Coordinator shakes hands with a survivor of Ebola in the Nyankunde Health Zone. This occurred during the official ceremony for the presentation of certificates attesting to the recovery of eight Ebola patients in this health zone. On this occasion, some of these survivors spoke to the press to share their experiences of recovery and to encourage adherence to various Ebola prevention measures. (These testimonies were broadcast on several radio stations.)

- **Support for other pillars :** Surveillance: investigation and pre-listing of contacts (Mudzimaria Health Zone/Bunia Health Zone). IPC: Preparing families to participate in the DHS in the event of community deaths (Bunia Health Zone, Bankoko and Bigo Health Zones). **Result:** DHS completed.

Community feedback : Complaints from patients at the CTE are not being properly addressed in terms of food, some are even leaving the hospital to go and look for food outside (recorded several times in Rwampara);

- The EDS teams are always late, and the young people take charge of the body and then challenge the Recommendations that call for recourse to EDS (recorded several times in Bunia)

4.8 Logistics and Security

- Delivery of IPC supplies, medicines and equipment to the health zones: BUNIA, RWAMPARA, DAMAS, KILO, MANGALA and KOMANDA
- Delivery of supplies to CH MATTYSEN, to the WHO Office
- Receiving and delivering cold chain equipment to the Grand LABO.

4.9. Security

The security situation in the city of Bunia (Bunia and Rwampara health zones) is relatively calm. However, the activity of armed groups in the territories of Djugu, Irumu, and Mambasa continues to limit humanitarian access in several affected or at-risk health zones.

4.10. PSEA (Prevention of Sexual Exploitation and Abuse)

In Goma and Beni : organization of awareness sessions by the IMC organization, for the second wave of community relays and community leaders – 155 people took part in these activities, including 71 women.

In Ituri : the PSEA pillar of the Ebola response organised a PSEA briefing session for a PSEA network member organisation, at the request of its manager – Subsequently, the Save the Children organisation conducted several PSEA briefing sessions in the Gety health zone – 106 people, including 51 women, participated in these briefings.

5. MAIN CHALLENGES

Challenge	Impact	Action required
No. 1 Reluctance to perform post-mortem sampling (swabs)	Confirmation delay; underestimation of cases	Intensive community engagement; involvement of religious leaders
2 Insufficient capacity in standardized CTEs despite the opening of Rwampara	Potential saturation of existing CTEs	Acceleration of the construction of planned CTEs
3 Weakness in contact tracing (overall rate 71.8%, target 95%)	Interruption of compromised transmission chains	Training, deployment and motivation of community relays
4 Supply shortage Essential Generic Medicines (MEG) in targeted HGRs	Weakening of overall care	A plea for MEG funding
5 Insufficient PCI equipment (PPE, DLM, chlorine) in North Kivu	Nosocomial risk	Urgent supplies
6 Low increase in alerts in the 3 provinces	Delay in detection of case	Strengthening community surveillance
7 Insufficient financial resources for all pillars	Slowdown in operations	Resource mobilization (gap of USD 21.5 million)

6. PERSPECTIVES AND PRIORITY ACTIONS (24-72h)

Priority Action		Due date
Responsible pillar		
	Training of service providers on the use of the RADIONE PCR	Laboratory Day 2
1 2	Briefing of service providers on Ebola virus disease (EVD) and tuberculosis control (TBC) surveillance in affected and high-risk areas	Monitoring/CREC Day 3
3	Briefing of community agents on Ebola surveillance and the SBC (Ituri province)	Monitoring/CREC Day 3
4	Supervised support from the Director General of INSP in the technical pillars	Coordination Continuous
5	Catch-up in contact tracing in Ituri remains weak.	Monitoring Day 7
6	Strengthening infection prevention and control (IPC) measures and staff training in Nyankunde and other nosocomial areas	PCI Day 2
7	Deployment of a standardized daily dashboard with KPIs per pillar	Coordination Day 3
8	Plan for catching up on samples awaiting analysis in North Kivu Laboratory	Day 2
9	North Kivu: discharge of the recovered case in Goma (Day 1), response to security incidents (ADF incursion), IPC training at Virunga General Hospital, CREC extension to households	Day 2
10	South Kivu: provision of 2 additional vehicles, installation of an internet kit in Miti-Murhesa, communication credits for the IT/ECZS	Day 2

Briefing of PSEA response stakeholders



Completion of the CTE CME Rwampara nursery



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